Trastuzumab emtansine (T-DM1) · metastatic

National Cancer Control Programme (NCCP) 00206 · Source reconciled; independent clinical, pharmacy and patient review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

T-DM1 3.6 mg/kg intravenous (IV) every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used according to the treatment plan. A port insertion is a separate procedure.

STEP 3

Treatment and observation

First infusion over 90 minutes, then observe for at least 90 minutes. Later infusions may take 30 minutes if tolerated, with at least 30 minutes observation.

STEP 4

After the visit

Contact the unit to rearrange a missed dose; the next schedule preserves a 3-week interval. This medicine does not routinely require a take-home pump.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

First infusion over 90 minutes, then observe for at least 90 minutes. Later infusions may take 30 minutes if tolerated, with at least 30 minutes observation.

Missed or delayed visit

Contact the unit to rearrange a missed dose; the next schedule preserves a 3-week interval.

Sources: NCCP 00206 v5 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient review pending.